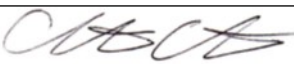


Guideline Title: Anxiety/Combative Patient Management

Guideline Number: GUID-3203-G006

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 2/7/2017, 6/8/2021	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To relieve anxiety, maximize patient care, and ensure safe transport of patient and transport team.
- II. **DEFINITIONS:**
- III. **DEPARTMENT GUIDELINE:**
 - A. Attempt verbal reassurance.
 - B. Apply appropriate extremity restraints to any patient observed to be agitated or with the potential to become agitated per *GUID3203-PR021 – Restraints*.
 - C. Apply & record end-tidal CO₂ (ETCO₂) monitoring on all patients requiring chemical/physical restraint.
 - D. Use the minimal number of physical and chemical restraints necessary to safely transport the patient.
 - E. Monitor for tachydysrhythmias, airway competence, oxygenation, and ventilation.
 - F. If patient does not respond to verbal reassurance, the following medications can be considered:
 - i. **Midazolam 0.05-0.1 mg/kg IV/IO**, may repeat every 3-5 minutes PRN, maximum dose 5 mg
 - ii. **Midazolam Intranasal**
 1. **Adults: 5 mg IN.** Administer half dose in each nostril.
 2. **Peds: 0.2 mg/kg.** Administer half dose in each nostril. Maximum dose 5 mg.
 - iii. **Haloperidol 5 mg IV/IM**

~ **OR** ~

- iv. **Ketamine**
 1. **Adults: 1 mg/kg slow IV/IO.** May repeat every 10 minutes PRN. Maximum total dose 200 mg.
 2. **Peds: 1 mg/kg slow IV/IO**, may repeat every 10 minutes PRN. Maximum total dose of 200 mg.
 3. Monitor for tachydysrhythmias

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4. Consider **Midazolam 0.05-0.1 mg/kg IV** for signs of emergence reaction (agitation).

IV. **DOCUMENTATION:**

- A. EMS Charts
- B. Documentation of least invasive measure before drug administration

V. **REFERENCES:**

- A. Lin J, Figuerado Y, Montgomery A, et al., Efficacy of ketamine for initial control of acute agitation in the emergency department: A randomized study. Am J Emerg Med 2021 44:306-11.